

RADIOLOGY ASSESSMENT REPORT

Patient Name:	Marcus Vance	Patient ID:	PT-10928
Age / Gender:	58 Y / Male	Accession No:	ACC-77491
Referred By:	Dr. Sarah Jenkins, MD	Study Date:	05-Jul-2026 11:15 AM
Modality:	X-Ray (CR)	Reported Date:	05-Jul-2026 04:30 PM

STUDY DETAILS

Examination: Chest PA and Lateral Views

Clinical Indication: Chronic productive cough, mild dyspnea, and low-grade pyrexia for 1 week.

DIAGNOSTIC IMAGES ATTACHED



[CHEST_PA_VIEW_IMAGE_ID_99214]

Embedded DICOM Reference Preview Placeholder

FINDINGS

Lungs: There is a distinct, patchy alveolar opacity / consolidation noted in the right lower lobe, consistent with localized airspace disease. The left lung field appears clear with normal vascular markings. No evidence of cavitary lesions or diffuse interstitial fibrosis.

Pleura: Right costophrenic angle shows mild blunting, highly suggestive of a small reactive pleural effusion. Left costophrenic and cardiophrenic angles are sharp and well-defined.

Mediastinum & Heart: The cardiomedial contour is within normal limits. Cardiac size is normal with no cardiomegaly. Hilar structures are clear. Trachea is midline.

Bones & Soft Tissues: Visualized thoracic skeleton shows no acute fractures or destructive osseous lesions. Soft tissues are unremarkable.

IMPRESSION

- Right lower lobe consolidation:** Highly indicative of acute lobar pneumonia.
- Mild right-sided pleural effusion:** Likely parapneumonic in origin.

Recommendation: Clinical correlation with inflammatory biomarkers and follow-up radiograph post-antibiotic treatment in 4-6 weeks to ensure complete resolution.

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